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### Project #054: Safeguarding the vulnerable: streamlining triage to rooming in the ED

Patrick Hanlon

*Henry Ford Health, phanlon1@hfhs.org*

Michelle Slezak

*Henry Ford Health, MSLEZAK1@hfhs.org*

Gina Hurst

*Henry Ford Health, ghurst1@hfhs.org*

Hilja Compian

*Henry Ford Health, hcompia1@hfhs.org*

Georgia Scholl

*Henry Ford Health, gscholl2@hfhs.org*

*See next page for additional authors*

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## Authors

Patrick Hanlon, Michelle Slezak, Gina Hurst, Hilja Compian, Georgia Scholl, Karra Robinson, and Namita Jayaprakash



# HENRY FORD HEALTH Safeguarding the Vulnerable: Streamlining Triage to Rooming in the ED

P. Hanlon M.D., G. Scholl RN MSN, M. Slezak, MD, G. Hurst, MD, Hilja Compian RN, Kara Robinson, RN, N. Jayaprakash MB BCH BAO, MRCEM

Department of Emergency Medicine, Henry Ford Hospital, Henry Ford Health, Detroit, Michigan

## Abstract

During times of ED crowding or surge it can be a challenging process to locate patients waiting for an assigned treatment space. De-centralized waiting areas with limitations to availability of overflow areas are major contributors. There are risks of erroneously marking patients as left without completing service (LWCS) and removing from the EMR when they are in fact still in the ED awaiting care. Quality assurance committee reviews and safety event reporting through the RLDatix system highlighted an upward trend of these events in 2024. Chart reviews identified that some of these patients are vulnerable with cognitive or physical impairments limiting their ability to advocate for themselves during long waits. An interdisciplinary team was formed, performed a root cause analysis (RCA) and developed a new triage workflow focused on safeguarding vulnerable patients presenting to the ED. The aim is to decrease by 10% the rate of patients registered and waiting to be seen in the ED following triage but prior to full evaluation, who are erroneously labeled as LWCS when in fact they remain in the ED.

## Problem

- Problem Description:** Patients labeled as left without completion of stay (LWCS) when in fact they remain in the Emergency Department and have not left.
- Identified causes:** fragmented locations of patients in triage, lack of effective communication between technicians and nursing staff, lack of protocol for triage process, patients unable to hear calls from staff that their treatment area was ready, or unable to move or respond to those calls.
- Specific goal:** Decrease the rate of patients incorrectly identified as LWCS who have not actually left the ED by 10% by the end of 2025

## Methods

- Root cause:** Crowded emergency department faced with disjointed waiting areas in combination with ineffective communication and processes that do not adequately account for patient factors/needs leads to patients erroneously listed as LWCS
- Measure:** Review of convenience sample of cases with disposition of LWCS and returned within 24 hours of initial ED presentation
- Study of interventions:** Weekly audits of process, feedback from triage staff, review of data at the end of 2025
- Aim:** 10% reduction in patients incorrectly marked as LWCS in the ED

## Triage Process for Vulnerable Patients

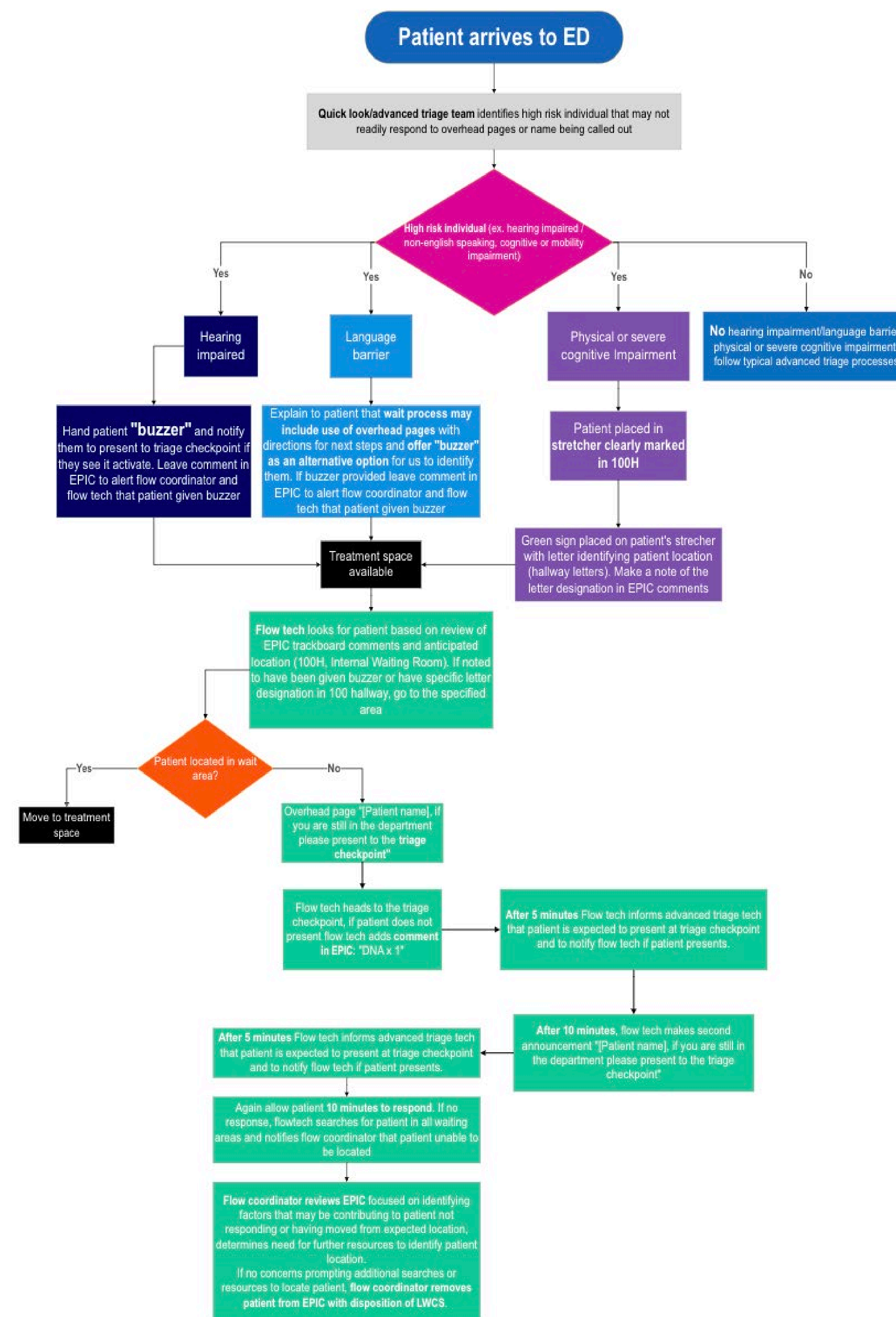


Figure 1. Triage algorithm for vulnerable patients presenting to the Emergency Department

## Proposed Process

- Early identification of vulnerable patients (cognitive or physical impairment, language barrier, etc) upon arrival to ED
- Use of audible devices such as Handheld buzzer for those identified as having hearing impairments or language barrier to notify them when their treatment area is ready
- Patients identified as cognitively or physically impaired placed in a more consistent waiting area on stretchers with a green sign and unique letter to identify them. Unique letter will be code entered into the ED comments section of track board to link EMR to location in ED
- Improve and protocolize communication amongst triage staff
- New signs marking triage checkpoint as area for patients to check in when questions arise, or their treatment area is ready for them

## Next Steps

- Launched 8/24/2025
- Strategy and tactics for implementation:
  - Patient stories to emphasize importance as a safety initiative
  - Weekly audits to assess fidelity to process
  - Just in time coaching
  - Collect data at the end of 2025 and compare to data from 2024 and evaluate for improvement
- Post intervention analysis of random sampling of LWCS with return within 24 hours to compare incidence rate

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- Nursing leadership team:** Hilja Compian, Karra Mattison, Georgia Scholl
- Funding:** Henry Ford Department of Emergency Medicine
- Ethical considerations:** This project seeks to improve the care of our most vulnerable patients. Our interdisciplinary team extensively discussed how the current triage process may not have been optimal for the care of vulnerable patients. We met many times over a course of months do perform a RCA to identify why this might be the case, and to identify a process to improve their care.
- No conflicts of interest to disclose

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